

COPD — Outpatient Management Clinical Reference

Diagnose → classify (GOLD 2023 ABE) → inhaler step-up → non-pharm → exacerbation. Adapted from TeachIM.org (CC BY-NC 4.0); GOLD 2024/2025 verified. Educational use — clinician-reviewed.

Diagnose

Required: post-bronchodilator **FEV1/FVC <0.70** on spirometry + symptoms/risk factors. No PFTs → no COPD dx.

Supportive PFTs: scooped flow-volume loop; ↑TLC, ↑RV (air trapping); ↓DLCO (emphysema). Post-BD FEV1 grades severity.

Workup: exposure hx; ABG if SpO2 <92%; **A1AT once in ALL** (esp. <45 or panlobular basal emphysema); CXR to exclude; CT if FEV1 <45%, sx out of proportion, or LCS.

pre-COPD = sx/structural, ratio ≥0.7. **PRISm** = preserved ratio, low volumes. Both can progress.

Classify — 2023 GOLD ABE

Group	Exac/yr	Symptoms
A	0–1	mMRC 0–1 / CAT <10
B	0–1	mMRC ≥2 / CAT ≥10
E	≥2 mod or ≥1 hosp	(any)

Key fork = exacerbation-prone or not (drives ICS).

Inhaler therapy

Group	First-line
A	One LABD (LABA or LAMA)
B	LABA + LAMA
E	LABA + LAMA (+ICS if eos ≥300)

Add ICS (→ triple) for exacerbaters + **eos ≥100**. **De-escalate ICS if recurrent PNA**. LABA-ICS has no clear GOLD role.

LABA = sx. LAMA = sx + fewer exac (>LABA). ICS = fewer exac if eos↑ (↑PNA). Step-up: roflumilast (FEV1<50%+CB), azithromycin (ex-smoker), LVRS/EBV/transplant. Technique + adherence every visit.

Non-pharm (efficacy)

Therapy	Benefit
Smoking cessation	Mortality + all domains
O2 (PaO2≤55, or ≤59+RHF)	Mortality, slows progression
Pulm rehab	Symptoms, exacerbations
Lung-cancer screen	Cancer mortality
Vaccines (flu/PCV/COVID)	Fewer exacerbations

Acute exacerbation

Cardinal sx: ↑dyspnea, ↑sputum volume, ↑sputum purulence. R/o mimics (PE, PNA, HF, ACS): CXR, D-dimer, BNP, ECG/trop.

Tx	Detail
Bronchodilator	SABA ± SAMA; MDI = neb if used right
Steroid	Prednisone 40 mg ×5 d (REDUCE)
Antibiotic	×5 d if ≥2 cardinal incl. purulence (or vent); per local resistance
NIPPV	pH <7.35 & PaCO2 >45

Hospitalize: resting dyspnea/↑O2/confusion, severe comorbidity, poor home support.

2024–25 GOLD updates

- Triple (LABA+LAMA+ICS) preferred **over LABA-ICS** when ICS indicated.
- Eos thresholds unchanged: ≥100 escalate; ≥300 initial ICS in Group E.
- **Dupilumab** (first biologic, FDA 9/2024): add to triple if exac + eos ≥300 + chronic bronchitis.
- **Ensifentrine** (Ohtuvayre, FDA 6/2024): nebulized PDE3/4 inhibitor.